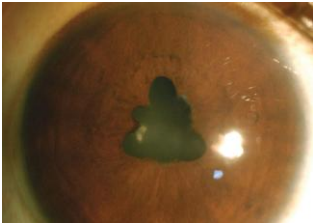


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QUESTIONS

- Q1. A patient gives a history of multiple episodes of painful red eye and decreased vision. On dilation, an abnormal pupil finding is seen. What is the likely diagnosis?



- a) Angle closure glaucoma
- b) Anterior uveitis
- c) Chronic conjunctivitis
- d) Ring ulcer

- Q2. A 6-year-old girl has uncorrected refractive error of +4.00 Dsph in the right eye and +0.50 Dsph in the left eye. Which of the following will be seen?

- a) Anisometropic amblyopia
- b) Sensory deprivation amblyopia
- c) Strabismus
- d) Diplopia

- Q3. All of the following are true for the condition shown in the fundus image except:



- a) Patient has a higher risk of retinal detachment
- b) Posterior staphyloma is seen
- c) Biconvex lens is used to correct the refractive error
- d) Fundus has a tessellated appearance

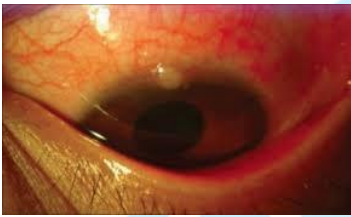
- Q4. A young child is brought with watering from the eyes. All of the following can be causes of watering except:

- a) Buphthalmos
- b) Congenital ptosis

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- c) Trichiasis
- d) Congenital NLDO

Q5. A young patient presents with redness and a nodular lesion in the eye. There is a family history of tuberculosis in the father. Slit-lamp findings are shown. What is the likely diagnosis?



- a) Vernal keratoconjunctivitis
- b) Mooren's ulcer
- c) Phlyctenular conjunctivitis
- d) Nodular episcleritis

Q6. A patient has recurrent painful red eye with blurred vision and photophobia. He also has backache with morning stiffness and X-ray evidence of sacroiliitis. Which of the following should be tested?

- a) RA factor
- b) HLA-B5
- c) HLA-B27
- d) Serum calcium levels

Q7. A 72-year-old lady has sudden marked loss of vision in the right eye, past minor stroke, RAPD and the shown fundus findings. What is the likely diagnosis?



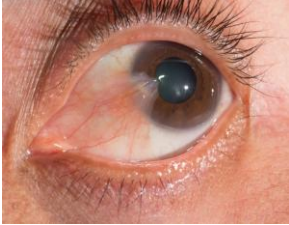
- a) Central retinal artery occlusion
- b) Cystoid macular edema
- c) Central retinal vein occlusion
- d) Serous retinal detachment

Q8. Which of the following is the true statement for the given condition?



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- a) Patient develops with-the-rule astigmatism
- b) Simple excision is the ideal treatment
- c) Patient presents with pain and discharge
- d) It is type 4 hypersensitivity reaction to tubercular antigen

Q9. A 42-year-old lady presents with progressively increasing painless proptosis in the left eye. Which diagnosis should be considered?

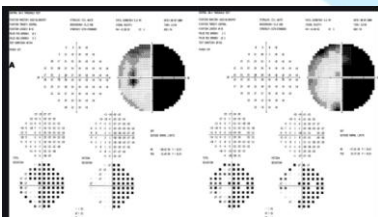
- a) Orbital cellulitis
- b) Orbital varices
- c) Thyroid ophthalmopathy
- d) Orbital lymphoma

Q10. What will be the glass prescription for the retinoscopy findings when done under atropine and at 1 m working distance?



- a) -3.0 Dsph / -1.0 Dcyl x 90 deg
- b) -1.0 Dsph / -2.0 Dcyl x 180 deg
- c) -2.0 Dcyl x 180 deg
- d) -3.0 Dsph / -1.0 Dcyl x 180 deg

Q11. What is the likely site of lesion in a patient with the field defect shown on perimetry?





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- a) Right optic tract
- b) Left occipital cortex
- c) Left optic tract
- d) Right optic nerve

Q12. A patient is not able to abduct his left eye and has diplopia on looking to the left side. What is the reason?

- a) Left lateral rectus palsy
- b) Right lateral rectus palsy
- c) Left medial rectus palsy
- d) Right medial rectus muscle palsy

Q13. Which of the following will NOT cause painless decrease in vision after cataract surgery?

- a) Cystoid macular edema
- b) Posterior capsular opacification
- c) Acute postoperative endophthalmitis
- d) Retinal detachment

Q14. A 45-year-old lady was treated for an acute attack of angle closure glaucoma in the right eye. What should be the next step of management?

- a) Laser iridotomy in right eye
- b) Laser iridotomy in left eye
- c) Laser iridotomy in both eyes
- d) Laser capsulotomy in right eye

Q15. A myopic patient is diagnosed with primary open-angle glaucoma and is a smoker. Which drug is the choice?

- a) Travoprost
- b) Timolol
- c) Acetazolamide
- d) Netarsudil

Q16. Which of the following is the MOST appropriate suture for skin closure of a contaminated wound?

- a) 3/0 Vicryl interrupted — absorbable, no suture removal
- b) Monofilament non-absorbable (Nylon/Prolene) — least tissue reaction, easily removed
- c) Braided silk — excellent knot security



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d) Staples — fastest closure, lowest infection rate in contaminated wounds

Q17. A 70-year-old man has a 2 cm basal cell carcinoma (BCC) on his right cheek. It has a pearly rolled edge with telangiectasia. What is the MOST appropriate treatment?

- a) Mohs micrographic surgery — first-line for all BCCs
- b) Simple excision with 4–5 mm clear margins — appropriate for low-risk BCC
- c) Radiotherapy — standard first-line for all facial BCCs
- d) Imiquimod cream only — effective for invasive BCC

Q18. A 48-year-old woman post-mastectomy and axillary clearance develops progressive swelling of the right arm. Limb volume difference is 25% compared to the contralateral arm. Which of the following is the CORRECT classification and initial management?

- a) Baker Grade III — compression garment + manual lymphatic drainage (MLD)
- b) International Society of Lymphology (ISL) Stage II lymphoedema — compression therapy + CDT (Complex Decongestive Therapy)
- c) Stage IV lymphoedema — surgical lymphovenous anastomosis immediately
- d) DVT of the axillary vein — urgent anticoagulation

Q19. A 55-year-old woman with ER+ breast cancer on anastrozole for 3 years has a bone scan showing new lesions in L3, L4, and ribs. No visceral metastases. Performance status 0. Next step?

- a) Switch to tamoxifen — anastrozole resistance developing
- b) CDK4/6 inhibitor (palbociclib/ribociclib) + fulvestrant — standard first-line for ER+ HER2- metastatic BC
- c) Urgent spinal decompression surgery — bone mets are a surgical emergency
- d) Palliative radiotherapy alone to bone metastases

Q20. A 19-year-old man sustains a closed head injury in a rugby match. He was briefly unconscious for 2 minutes then recovered. In the ED his GCS is 15. He has a headache and reports amnesia for the event. Over the next 3 hours his GCS drops to 13, then to 9. CT head shows a biconvex (lens-shaped) hyperdense collection in the right temporal region with 8mm midline shift. What is the diagnosis, and how does the GCS trajectory guide the TIMING of surgical intervention?

- a) Acute subdural haematoma — urgent craniotomy only if GCS drops below 3
- b) Extradural haematoma (EDH) — the classic lucid interval followed by GCS deterioration is the hallmark; GCS deterioration is a surgical emergency requiring immediate craniotomy/burr hole
- c) Diffuse axonal injury — GCS trajectory is irrelevant; MRI is required before any decision
- d) Intracerebral haemorrhage — conservative management with antihypertensives



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- Q21. A 45-year-old man sustains 65% TBSA full thickness burns in an industrial explosion. He is resuscitated and haemodynamically stable. Donor site availability is severely limited (unburned skin = 35% TBSA). The burns team plan a staged reconstruction. Which of the following CORRECTLY describes the management strategy for massive burns with limited donor sites?
- a) Meshed SSG at 1:6 ratio over all burn wounds in a single operation — donor sites re-harvested immediately
 - b) Staged approach: temporary biological dressings (cadaveric allograft or Biobrane) to close wounds temporarily → serial SSG harvest with re-harvesting donor sites every 10–14 days → dermal substitute (Integra/Matriderm) for areas requiring optimal functional/aesthetic outcome
 - c) Permanent cadaveric allograft — lifelong immunosuppression to prevent rejection
 - d) Conservative management with silver dressings only — skin grafting not feasible for >60% TBSA burns

- Q22. A 24-year-old chronic smoker presents with:

Burning pain in feet

Migratory thrombophlebitis

Digital ischemia

Angiography demonstrates:

Corkscrew collaterals

Distal vessel involvement

Most likely diagnosis?

- a) Takayasu arteritis
- b) Atherosclerotic PAD
- c) Buerger disease
- d) Polyarteritis nodosa

- Q23. A 72-year-old male presents with:

Pulsatile abdominal mass

Back pain

Hypotension

Bedside ultrasound demonstrates:

8 cm infrarenal abdominal aortic aneurysm

Most likely diagnosis?



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- a) Aortic dissection
- b) Ruptured abdominal aortic aneurysm
- c) Mesenteric ischemia
- d) Iliac aneurysm

Q24. A 65-year-old man presents with a rapidly enlarging parotid swelling for 6 months. He reports severe pain radiating to the ear and progressive deviation of the angle of the mouth.

Examination reveals:

Hard fixed parotid mass

Facial nerve palsy

Enlarged cervical nodes

MRI shows:

Deep lobe involvement

Perineural spread

Which clinical finding most strongly suggests malignancy?

- a) Long duration of swelling
- b) Deep lobe involvement
- c) Facial nerve palsy
- d) Male gender

Q25. A 40-year-old man presents with a painless swelling of the right parotid gland for 2 years. USS shows a well-defined, homogeneous, 3 cm mass in the superficial lobe. FNA: benign pleomorphic adenoma. What is the MOST appropriate management?

- a) Observation — all parotid tumours are benign
- b) Superficial parotidectomy with facial nerve preservation
- c) Total parotidectomy with sacrifice of the facial nerve
- d) Radiotherapy — preferred for all salivary gland tumours

Q26. A 55-year-old man presents with:

Multiple liver metastases

Ileal primary carcinoid

Why did carcinoid syndrome appear?





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- a) Tumor size >2 cm
- b) Liver metastases bypass hepatic metabolism
- c) Ileal location
- d) Elevated CEA

Q27. A 37-year-old woman presents with recurrent episodes of sweating, tremors, confusion, and blurred vision, especially after fasting. Symptoms improve immediately after oral glucose. During supervised fasting, plasma glucose is 38 mg/dL with inappropriately elevated insulin and C-peptide.

Most likely diagnosis?

- a) Glucagonoma
- b) Insulinoma
- c) VIPoma
- d) Somatostatinoma

Q28. A 63-year-old man presents with painless jaundice. CT shows pancreatic head mass. During staging laparoscopy, small peritoneal deposits are found.



Best management?

- a) Proceed with Whipple procedure
- b) Palliative biliary bypass/stenting and systemic therapy
- c) Distal pancreatectomy
- d) Total gastrectomy

Q29. A 32-year-old man presents 6 months after a pelvic fracture-associated urethral injury with inability to void, poor stream, and recurrent UTIs. Flexible cystoscopy from a suprapubic track shows a blind-ending urethra. RUG shows complete obliteration of the bulbomembranous urethra over a 2cm length. What is the DEFINITIVE treatment?

- a) Regular urethral dilation — long-term management for post-traumatic strictures
- b) Direct visual internal urethrotomy (DVIU) — incise the stricture endoscopically
- c) Anastomotic bulbomembranous urethroplasty (Pelvic urethral reconstruction) — excision of fibrotic segment + spatulated end-to-end anastomosis



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d) Permanent suprapubic catheter — complex strictures cannot be repaired

Q30. In the TEP (Totally Extraperitoneal) laparoscopic inguinal hernia repair, the dissection is performed in which anatomical space?

- a) Intraperitoneal space — same as TAPP
- b) Preperitoneal space — between the peritoneum and transversalis fascia; peritoneal cavity NOT entered
- c) Subserosal space
- d) Retroperitoneal space lateral to the kidney

Q31. A 55-year-old man undergoes Lichtenstein tension-free mesh repair for a right indirect inguinal hernia. 6 months later he presents with severe burning inguinodynia (groin pain). He is otherwise well with no recurrence. What is the MOST likely cause?

- a) Mesh infection requiring removal
- b) Inguinodynia from entrapment of the ilioinguinal nerve (or iliohypogastric/genitofemoral nerve) — most common cause of chronic pain after Lichtenstein repair
- c) Hernia recurrence
- d) Femoral nerve injury

Q32. A 50-year-old man undergoes PCNL for a 25mm left renal pelvis stone. The procedure is completed successfully. 4 hours post-operatively he develops hypotension (BP 80/50), tachycardia (HR 135), and his nephrostomy tube output is 300mL of blood-stained fluid in 1 hour. CT angiography shows active bleeding from a pseudoaneurysm of a segmental renal artery at the puncture site. What is the MOST appropriate management?

- a) IV fluid resuscitation and observation — nephrostomy site bleeding always resolves
- b) Selective renal artery angioembolisation (SAE) of the pseudoaneurysm
- c) Emergency open nephrectomy
- d) Clamp the nephrostomy tube — tamponade effect will stop bleeding

Q33. A 34-year-old pregnant woman (28 weeks gestation) presents with severe right loin pain and haematuria. USS shows mild right hydronephrosis. CT KUB is not performed. MRI urogram is inconclusive. She is afebrile and haemodynamically stable. Urine culture is negative. What is the MOST appropriate initial management?

- a) ESWL — safe in the second trimester
- b) Conservative management — analgesia (paracetamol ± opioids), hydration, positional changes; most stones pass spontaneously; USS monitoring
- c) CT KUB — radiation risk acceptable in 28 weeks gestation
- d) Immediate ureteroscopy under general anaesthesia



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- Q34. A 40-year-old woman with multiple small gallstones (<5mm) undergoes laparoscopic cholecystectomy. During port removal, a stone is dropped into the abdomen. What is the MOST appropriate action?
- Convert to open surgery immediately to retrieve all stones
 - Attempt retrieval laparoscopically; if unsuccessful, document and inform patient — monitor for complications
 - Leave stones — they will be absorbed by the peritoneum harmlessly
 - Irrigate extensively with saline — stones dissolve in saline
- Q35. A 60-year-old man has acute acalculous cholecystitis (AAC) in the ICU following major abdominal surgery. Which of the following risk factors is MOST strongly associated with AAC?
- Female sex and obesity
 - Critical illness, prolonged fasting, TPN, major surgery, opioids
 - Oral contraceptive pill use
 - Rapid weight loss after bariatric surgery
- Q36. Which of the following is TRUE regarding the cystic artery?
- Arises from the left hepatic artery in 80% of cases
 - Arises from the right hepatic artery within Calot's triangle in the majority of cases
 - Is always a single artery
 - Arises directly from the common hepatic artery in most cases
- Q37. Which radiological investigation has the HIGHEST sensitivity for detecting common bile duct stones in a patient with suspected choledocholithiasis?
- Transabdominal ultrasound (TUS)
 - CT abdomen with contrast
 - MRCP (Magnetic Resonance Cholangiopancreatography)
 - Endoscopic Ultrasound (EUS)
- Q38. A 55-year-old woman with known primary biliary cholangitis (PBC) presents with jaundice, pruritus, and fatigue. ALP is 800 U/L, GGT 600 U/L, bilirubin 45 $\mu\text{mol/L}$. Anti-mitochondrial antibody (AMA-M2) is strongly positive. Which statement about PBC is CORRECT?
- PBC predominantly affects men aged 30–45
 - PBC is associated with ulcerative colitis in 70% of cases
 - PBC is an autoimmune condition affecting small intrahepatic bile ducts; AMA-M2 is diagnostic; UDCA is first-line treatment
 - ERCP shows beading of intra and extrahepatic ducts (pathognomonic)



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Q39. A 50-year-old patient presents with gastric outlet obstruction. CT shows a large gallstone impacted in the duodenum after passing through a cholecystoduodenal fistula.

Diagnosis?

- a) Mirizzi syndrome
- b) Bouveret syndrome
- c) Zollinger-Ellison syndrome
- d) Boerhaave syndrome

Q40. A 42-year-old female undergoes laparoscopic cholecystectomy.

During surgery:

Dense adhesions

Severe Calot triangle inflammation

Surgeon is unable to clearly identify anatomy.

What is the safest next step?

- a) Continue aggressive dissection
- b) Divide cystic structures blindly
- c) Convert to open surgery
- d) Abort procedure and close

Q41. A chronic alcoholic presents with:

Steatorrhea

Weight loss

Pancreatic calcification

Dilated pancreatic duct

Most appropriate surgery?

- a) Whipple procedure
- b) Distal pancreatectomy
- c) Puestow procedure
- d) Total pancreatectomy



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- Q42. A 55-year-old obese man presents with a 9mm right lower pole renal stone on CT. He is asymptomatic and afebrile. eGFR is 72 mL/min. He has a pacemaker in situ. Which treatment modality is CONTRAINDICATED in this patient?
- Flexible ureteroscopy and laser lithotripsy (FURS/RIRS)
 - Percutaneous nephrolithotomy (PCNL)
 - ESWL (extracorporeal shock wave lithotripsy)
 - Observation with serial imaging
- Q43. A 38-year-old woman is found to have a 6mm right ureteric stone at the level of L3 (upper ureter) on CT KUB. She is haemodynamically stable, afebrile, and has adequate pain control. eGFR normal. Contralateral kidney normal. She is keen to avoid surgery. Which statement about ESWL for this stone is CORRECT?
- ESWL is contraindicated for upper ureteric stones
 - ESWL is appropriate for upper ureteric stones $\leq 10\text{mm}$ — equivalent efficacy to ureteroscopy; requires prone positioning with posterior approach
 - ESWL stone-free rates for upper ureteric stones are 90% after a single session
 - ESWL requires general anaesthesia in all cases
- Q44. A 30-year-old man presents with bilateral renal stones since age 15. He is on allopurinol but continues to form stones. Stone analysis: 2,8-dihydroxyadenine (2,8-DHA). Genetic testing shows homozygous APRT (adenine phosphoribosyltransferase) gene mutation. Which of the following is the MOST specific treatment?
- Potassium citrate — alkalinise urine
 - Allopurinol — reduces purine synthesis; already on it, increase dose
 - Febuxostat — xanthine oxidase inhibitor, superior to allopurinol for APRT deficiency
 - High fluid intake + dietary purine restriction + allopurinol optimisation — 2,8-DHA stones are radiolucent and require chemolysis
- Q45. A 65-year-old man with a right ureteric stone (8mm, mid-ureter) undergoes ureteroscopy and laser lithotripsy. Post-procedure, a JJ stent is placed. 4 weeks later he returns with dysuria, frequency, urgency, and haematuria. His urine culture is sterile. KUB X-ray shows the stent in correct position. What is the MOST likely diagnosis and management?
- Ureteric stone recurrence — repeat CT KUB urgently
 - JJ stent-related lower urinary tract symptoms (LUTS) — alpha-blocker (tamsulosin) + anticholinergic (solifenacin); remove stent at planned interval (4–6 weeks)
 - Urinary tract infection — empirical antibiotics
 - Bladder tumour — urgent cystoscopy



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- Q46. A 55-year-old woman with a 15mm right lower pole renal stone is being counselled about treatment options. The stone is radio-opaque on plain X-ray. She has no coagulopathy, no infection, and normal renal function. Which statement about lower pole stones specifically is CORRECT, and which treatment is PREFERRED?
- a) ESWL has the same efficacy for lower pole as for renal pelvis stones — ESWL is preferred
 - b) Lower pole stones have reduced ESWL stone-free rates due to gravity impairing fragment clearance downward; flexible ureteroscopy (FURS) or PCNL preferred over ESWL for lower pole stones $\geq 10\text{mm}$
 - c) All lower pole stones require PCNL regardless of size
 - d) Lower pole stones always require open surgery due to anatomical difficulty
- Q47. A 52-year-old man has been investigated for recurrent calcium oxalate stones (4 episodes in 3 years). His 24-hour urine collection shows: urine volume 900mL/24h, calcium 380mg/24h, oxalate 38mg/24h (normal), uric acid 950mg/24h, citrate 120mg/24h (low, normal $>320\text{mg}$). Serum calcium and PTH are normal. Which combination of interventions is MOST appropriate for his stone prevention?
- a) Low calcium diet + allopurinol
 - b) Increase fluid intake (target urine output $>2\text{L/day}$) + thiazide diuretic (indapamide) for hypercalciuria + potassium citrate for hypocitraturia + dietary modification
 - c) Potassium citrate only — hypocitraturia is the only abnormality
 - d) Allopurinol alone — hyperuricosuria is the primary driver
- Q48. A 32-year-old man presents with classical acute appendicitis (Alvarado score 9, CT confirmed). He is fit and well. The surgical team plans laparoscopic appendectomy. He asks whether antibiotics alone could treat his appendicitis without surgery. Which of the following BEST represents the current evidence?
- a) Antibiotics alone are equally effective as appendectomy in all cases — surgery should be abandoned
 - b) Antibiotics alone (non-operative management) can treat uncomplicated (non-perforated) appendicitis in 70–72% at 1 year; 30% require appendectomy within 1 year; recurrence rate $\sim 40\%$ at 5 years — patient must be fully informed
 - c) Antibiotics are only effective for appendicitis in children — not adults
 - d) Non-operative management carries a higher 30-day mortality than appendectomy and should never be offered
- Q49. A 60-year-old woman presents with postmenopausal bleeding for 1 month. She attained menopause 10 years ago. There is no history of hormone replacement therapy. General and pelvic examination are unremarkable.

Transvaginal ultrasound shows an endometrial thickness of 6 mm with no obvious focal mass.

What is the most appropriate next step in management?

- a) Reassurance and discharge



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- b) Vaginal estrogen therapy
- c) Endometrial biopsy (Pipelle)
- d) Hysteroscopy directly

Q50. A 58-year-old postmenopausal woman presents with complaints of intermittent vaginal bleeding for the past 2 weeks. She attained menopause 8 years ago. There is no history of hormone therapy use. On examination, no obvious cervical lesion is seen.

Transvaginal ultrasound reveals endometrial thickness of 3 mm with no focal lesion.

What is the most appropriate next step in management?

- a) Endometrial biopsy (Pipelle)
- b) Hysteroscopy
- c) Start vaginal estrogen therapy and reassure
- d) Immediate hysterectomy

Q51. A 56-year-old postmenopausal woman presents with hot flashes and decreased libido. She is concerned about the risk of breast and endometrial cancer with conventional HRT. The doctor prescribes a drug that has estrogenic, progestogenic, and androgenic effects and does not cause endometrial hyperplasia.

Which of the following is the drug of choice?

- a) Conjugated estrogen
- b) Medroxyprogesterone
- c) Tibolone
- d) Tamoxifen

Q52. A woman with molar pregnancy develops palpitations, tremors, and heat intolerance. Laboratory investigations reveal elevated thyroid hormones.

What is the most likely cause?

- a) Autoimmune thyroiditis
- b) Pituitary adenoma
- c) hCG-mediated thyroid stimulation
- d) Thyroid hormone resistance



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Q53. 32-year-old multigravida at 33 weeks gestation presents with sudden onset painless bright red vaginal bleeding. She had similar episodes previously. Uterus is soft and non-tender. Fetal heart rate is normal.

What is the most appropriate next step?

- a) Perform per vaginal examination
- b) Immediate induction of labor
- c) Transvaginal ultrasound for placental localization
- d) Emergency cesarean section

Q54. A patient with APH has a soft, non-tender uterus and fetal parts are easily palpable.

Which additional finding is most likely?

- a) Concealed hemorrhage
- b) Severe abdominal pain
- c) Transverse lie
- d) Board-like rigidity

Q55. A 35-year-old multiparous woman with chronic pelvic inflammatory disease presents with severe chronic pelvic pain not responding to antibiotics. She has completed her family.

What is the most appropriate definitive management?

- a) Supracervical hysterectomy with bilateral salpingectomy
- b) Radical hysterectomy
- c) Total abdominal hysterectomy with bilateral salpingo-oophorectomy
- d) Supracervical hysterectomy with tuboplasty

Q56. A woman undergoes successful vesicovaginal fistula repair. What advice should be given during discharge?

- a) Sexual abstinence for 3 months and avoid pregnancy for 1 year
- b) Sexual abstinence for 3 weeks and avoid pregnancy for 6 months
- c) Sexual abstinence for 6 weeks and avoid pregnancy for 1 year
- d) Sexual abstinence for 6 months and avoid pregnancy for 6 years

Q57. A 28-year-old woman at 35 weeks and 3 days gestation has oligohydramnios with AFI 5 cm and absent end-diastolic flow in the umbilical artery Doppler.

What is the most appropriate management?



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- a) Daily Doppler and wait
- b) Corticosteroids and delay till 36 weeks
- c) Delivery after 37 weeks
- d) Immediate delivery

Q58. A 28-year-old G3P2L2 woman at 36 weeks gestation is found to have a transverse lie. Liquor is adequate and there are no contraindications to vaginal delivery.

What is the best management?

- a) Cesarean section
- b) External cephalic version
- c) Expectant management
- d) Induction of labor

Q59. A gravida 4 woman in labor undergoes amniotomy. The liquor appears golden yellow.

What is the most likely cause?

- a) Intra-amniotic infection
- b) Rh isoimmunization
- c) Chorioamnionitis
- d) Postmaturity

Q60. A pregnant woman undergoes biophysical profile testing.

Which of the following is a normal parameter?

- a) AFI 1–5 cm
- b) One episode of FHR acceleration >15 bpm
- c) Three or more discrete body or limb movements
- d) Two episodes of limb extension only

Q61. A 14-year-old phenotypic female presents with primary amenorrhea. Examination shows Tanner stage IV breasts, absent pubic hair, blind vaginal pouch, and inguinal mass. Ultrasound shows absent uterus and ovaries. Karyotype is 46,XY.

What is the best next step in management?

- a) Elective gonadectomy
- b) Estrogen therapy
- c) Growth hormone therapy



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d) Reassurance only

Q62. Match the following ovarian tumors with their tumor markers:

Yolk sac tumor	a. CA-125
Dysgerminoma	b. AFP
Choriocarcinoma	c. LDH
Epithelial ovarian tumor	d. Beta-hCG

- a) 1-b, 2-c, 3-d, 4-a
- b) 1-c, 2-b, 3-d, 4-a
- c) 1-b, 2-c, 3-a, 4-d
- d) 1-c, 2-b, 3-a, 4-d

Q63. A 25-year-old man underwent semen analysis.

Findings:

- Sperm concentration: 16 million/mL
- pH: 7.5
- Volume: 2 mL
- Agglutination absent
- 60% normal morphology
- 60% motile sperms

Diagnosis?

- a) Oligospermia
- b) Normospermia
- c) Asthenospermia
- d) Teratospermia

Q64. Arrange the steps of dilatation and curettage in sequence:

Serial dilatation

Curettage

Uterine sound

Assessment of uterine size and direction

- a) 4-3-2-1



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- b) 4-2-3-1
- c) 4-3-1-2
- d) 4-1-2-3

Q65. A pregnant lady delivered her baby unattended while on the way to the hospital. On examination, a perineal tear with disruption of the anal sphincter is noted. This tear is classified as:

- a) 1st degree
- b) 2nd degree
- c) 3rd degree
- d) 4th degree

Q66. Arrange the following cardinal movements of labor in sequence:

- A. Crowning
- B. Engagement
- C. Restitution
- D. External rotation

- a) B, A, C, D
- b) B, A, D, C
- c) B, C, A, D
- d) B, D, A, C

Q67. A woman at 40 weeks gestation is in second stage of labor for 2 hours. PV examination shows fetal station +3, sagittal suture in AP diameter and caput +. Fetal heart rate is 70/min for the last 20 minutes. What is the most appropriate method of delivery?

- a) Outlet forceps
- b) Ventouse delivery
- c) Mid cavity forceps
- d) Low cavity forceps

Q68. What is the correct dose of antenatal steroids for fetal lung maturity?

- a) Betamethasone 12 mg 12 hours apart $\times$ 4 doses
- b) Dexamethasone 6 mg 12 hours apart $\times$ 4 doses
- c) Betamethasone 6 mg 12 hours apart $\times$ 4 doses
- d) Dexamethasone 12 mg 24 hours apart $\times$ 2 doses



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- Q69. A pregnant Rh-negative woman has a negative indirect Coombs test. When should prophylactic Anti-D be administered?
- a) 28 weeks
 - b) 24 weeks
 - c) 28 weeks and monthly thereafter
 - d) 24 weeks and monthly thereafter
- Q70. During cesarean section for placenta previa in a woman with two previous cesarean deliveries, the placenta fails to separate and brisk hemorrhage occurs. The bladder appears densely adherent to the lower uterine segment. Most likely diagnosis?
- a) Placenta accreta
 - b) Placenta increta
 - c) Placenta percreta
 - d) Circumvallate placenta
- Q71. A 24-year-old primigravida at 12 weeks undergoes combined screening. NT is increased. PAPP-A is low and free β -hCG is elevated. Which chromosomal abnormality is most likely?
- a) Trisomy 18
 - b) Trisomy 13
 - c) Trisomy 21
 - d) Turner syndrome
- Q72. A 34-year-old woman with monochorionic diamniotic twins is found to have severe polyhydramnios in one sac and oligohydramnios in the other. The recipient twin has a distended bladder while the donor twin has an invisible bladder. Most likely diagnosis?
- a) Twin anemia-polycythemia sequence
 - b) Twin-to-twin transfusion syndrome
 - c) Selective fetal growth restriction
 - d) Acardiac twin
- Q73. A primigravida at 39 weeks is in labor. The fetal head is visible at the introitus without separating the labia. Fetal bradycardia of 70/min is noted. The most appropriate mode of delivery is:
- a) Cesarean section
 - b) Ventouse delivery
 - c) Outlet forceps delivery
 - d) Low forceps delivery



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Q74. A woman with previous lower-segment cesarean section is undergoing trial of labor. Suddenly, contractions cease, fetal heart rate drops, and the presenting part recedes. Diagnosis?

- a) Cord prolapse
- b) Placental abruption
- c) Uterine rupture
- d) Shoulder dystocia

Q75. A 48-year-old woman presents with heavy menstrual bleeding. Histopathology shows complex atypical endometrial hyperplasia. Risk of progression to endometrial carcinoma is highest with:

- a) Simple hyperplasia without atypia
- b) Complex hyperplasia without atypia
- c) Simple hyperplasia with atypia
- d) Complex atypical hyperplasia

Q76. A 22-year-old woman presents with fatigue, intermittent fever, joint pain involving both wrists and knees, and a facial rash that worsens after sun exposure. She also reports painless oral ulcers and hair loss over the past few months. On examination, a malar ("butterfly") rash is noted across the cheeks and bridge of the nose. Laboratory investigations reveal anemia, proteinuria, and a positive antinuclear antibody (ANA) test.

Which of the following autoantibodies is the most specific for the diagnosis of this patient's condition?

- a) Anti-histone antibody
- b) Anti-Ro (SS-A) antibody
- c) Anti-double-stranded DNA (anti-dsDNA) antibody
- d) Rheumatoid factor (RF)

Q77. A 42-year-old woman presents with pain, swelling, and morning stiffness of the small joints of both hands for the past 8 months. The stiffness lasts for more than 1 hour and improves with activity. On examination, there is symmetrical involvement of the metacarpophalangeal (MCP) and proximal interphalangeal (PIP) joints. X-ray of the hands shows periarticular osteopenia and marginal erosions.

To confirm the diagnosis, a serological test with the highest specificity is ordered.

Which of the following antibodies is most specific for the above disorder?

- a) Rheumatoid factor (RF)
- b) Anti-nuclear antibody (ANA)
- c) Anti-cyclic citrullinated peptide (Anti-CCP) antibody
- d) Anti-double-stranded DNA (Anti-dsDNA) antibody



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Q78. A 50-year-old male presents with progressive breathlessness and dry cough for the past 2 years. He also complains of tightness of skin over his fingers, difficulty swallowing, and episodes of finger discoloration on exposure to cold. He has worked in a sandstone quarry for over 25 years with significant exposure to silica dust. On examination, there is sclerodactyly, digital ulcers, and reduced oral aperture. Chest auscultation reveals fine bibasilar crackles.

Investigations reveal:

Chest X-ray: Multiple nodular opacities predominantly in the upper lobes

HRCT chest: Interstitial fibrosis

Pulmonary function test: Restrictive pattern

Which of the following combinations best establishes the diagnosis of Erasmus syndrome in this patient?

- a) Silica exposure + pulmonary nodules + Anti-double stranded DNA positivity
- b) Silica exposure + interstitial lung disease + Anti-topoisomerase I (anti-Scl-70) positivity
- c) Silica exposure + cavitary lung lesions + Anti-centromere antibody positivity
- d) Silica exposure + pleural effusion + Anti-histone antibody positivity

Q79. A 21-year-old man is evaluated for persistent mild jaundice noticed since adolescence. He has no history of abdominal pain, weight loss, pruritus, alcohol use, or blood transfusions. Physical examination is unremarkable except for mild icterus. Laboratory investigations reveal:

Total bilirubin: 4.2 mg/dL

Predominantly conjugated hyperbilirubinemia

Normal AST, ALT, and ALP

Negative viral hepatitis markers

A liver biopsy demonstrates coarse dark brown-black pigment deposition in hepatocytes, giving the liver a characteristic black appearance.

A defect in which of the following genes is most likely responsible for this condition?

- a) UGT1A1
- b) ATP7B
- c) ABCC2 (MRP2)
- d) HFE

Q80. A 27-year-old man presents with fatigue, nausea, right upper quadrant discomfort, and dark urine for 5 days. He recently had unprotected sexual contact.

Labs show: ALT 950 U/L, AST 720 U/L, total bilirubin 3.1 mg/dL.



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Hepatitis B serology:**

HBsAg: Positive

Anti-HBc IgM: Positive

HBeAg: Positive

Anti-HBs: Negative

What is the most likely diagnosis?

- a) Acute Hepatitis B infection
- b) Chronic active Hepatitis B
- c) Window period of Hepatitis B
- d) Immunity due to vaccination

Q81. A 42-year-old woman presents with progressive fatigue, generalized itching, and mild jaundice for the past several months. Examination reveals excoriation marks over the limbs. Laboratory investigations show:

Markedly elevated alkaline phosphatase (ALP) and GGT

Mildly elevated bilirubin

Positive anti-mitochondrial antibody (AMA)

Liver biopsy demonstrates chronic nonsuppurative destruction of small intrahepatic bile ducts. What is the most likely diagnosis?

- a) Primary Sclerosing Cholangitis
- b) Primary Biliary Cholangitis
- c) Autoimmune Hepatitis
- d) Drug-induced cholestasis

Q82. A 17-year-old boy is brought by his parents due to progressive mood changes, irritability, and declining academic performance over the past year. His teachers report new-onset difficulty with concentration and episodes of inappropriate laughter. Over the past 3 months, he has also developed slurred speech, difficulty writing, and occasional tremors in both hands, especially when reaching for objects.

He denies alcohol or drug use. There is no family history of neurologic disorders. On physical examination, he appears mildly jaundiced. His abdomen is soft but shows palpable hepatomegaly. Neurologic examination reveals dysarthria, a wing-beating tremor, and mild rigidity of the upper limbs.

Routine labs show:



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AST: 118 U/L

ALT: 102 U/L

Bilirubin mildly elevated

INR slightly prolonged

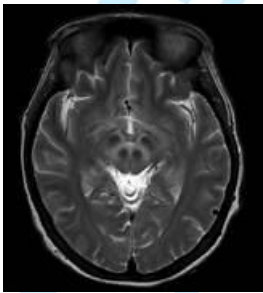
Further testing reveals:

Serum ceruloplasmin: significantly low

24-hour urinary copper: markedly elevated

A slit-lamp eye examination demonstrates golden-brown rings encircling the cornea.

Which diagnosis best explains this patient's presentation?



- a) Early-onset Huntington disease
- b) Wilson disease
- c) Autoimmune hepatitis
- d) Hereditary hemochromatosis

Q83. A 22-year-old man presents with a history of recurrent episodes of flank pain and passage of kidney stones since adolescence. Over the past few months, he has also experienced generalized weakness and intermittent muscle cramps. There is no history of diabetes mellitus or chronic kidney disease.

On examination, his blood pressure is normal. Laboratory investigations reveal:

Serum bicarbonate (HCO_3^-): 17 mEq/L

Serum potassium: 2.9 mEq/L

Arterial blood gas: normal anion gap metabolic acidosis

Urine pH: 6.5

Ultrasound of the abdomen demonstrates bilateral nephrocalcinosis with multiple renal calculi. Which of the following is the most likely diagnosis?



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- a) Type I (Distal) Renal Tubular Acidosis
- b) Type II (Proximal) Renal Tubular Acidosis
- c) Type IV Renal Tubular Acidosis
- d) Gitelman syndrome

Q84. A 6-year-old boy is brought to the pediatric clinic with progressive facial puffiness and swelling of both legs for the past 1 week. His parents report that the swelling is more prominent around the eyes in the morning. There is no history of hematuria, hypertension, rash, or recent sore throat.

Laboratory investigations reveal:

Urine protein: 4+

24-hour urinary protein: 4.5 g/day

Serum albumin: 2.0 g/dL

Serum cholesterol: 320 mg/dL

Serum creatinine: Normal

Light microscopy of the renal biopsy appears essentially normal. Electron microscopy shows diffuse effacement of podocyte foot processes.

Which of the following is the most likely diagnosis?

- a) Focal segmental glomerulosclerosis
- b) Membranous nephropathy
- c) Minimal change disease
- d) Post-streptococcal glomerulonephritis

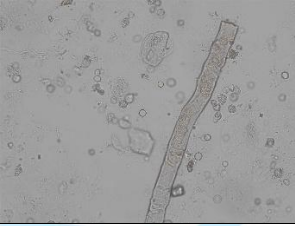
Q85. Match the casts with the conditions they are found in:

Casts	
1. 	a- Nephritic syndrome
2. 	b- CKD



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3.		c- ATN
4.		d- PN

- a) 1-d, 2-c, 3-a, 4-b
- b) a, 2- d, 3- b, 4- c
- c) 1-a, 2- d, 3-c, 4-b
- d) 1-d, 2- a, 3- c, 4- b

Q86. A 56-year-old man with a 10-year history of chronic kidney disease presents with generalized bone pain, proximal muscle weakness, and persistent itching. He is on regular treatment for hypertension and diabetes. Examination reveals bony tenderness over the ribs and long bones.

Laboratory investigations show:

- Serum calcium: 7.6 mg/dL (low)
- Serum phosphate: 6.8 mg/dL (high)
- Serum creatinine: 4.5 mg/dL
- Intact parathyroid hormone (PTH): markedly elevated
- Serum 1,25-dihydroxyvitamin D: decreased

X-ray of the hands shows subperiosteal bone resorption. Which of the following is the most likely diagnosis?

- a) Primary hyperparathyroidism
- b) Secondary hyperparathyroidism
- c) Tertiary hyperparathyroidism
- d) Hypoparathyroidism

Q87. A 42-year-old woman presents with poorly controlled hypertension, recurrent episodes of muscle weakness, and occasional muscle cramps for the past 6 months. She is not taking diuretics. On examination, her blood pressure is 170/100 mmHg.

Laboratory investigations reveal:

- Serum potassium: 2.8 mEq/L
- Serum bicarbonate: 32 mEq/L



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- Plasma aldosterone concentration: elevated
- Plasma renin activity: suppressed
- Aldosterone-to-renin ratio: markedly increased

CT scan of the abdomen shows a 2-cm unilateral adrenal cortical nodule. Which of the following is the most likely diagnosis?

- a) Secondary hyperaldosteronism due to renal artery stenosis
- b) Conn syndrome
- c) Liddle syndrome
- d) Cushing syndrome

Q88. A 36-year-old man presents with recurrent episodes of renal stones, constipation, and generalized bone pain for the past 2 years. He also complains of recurrent peptic ulcer disease despite prolonged proton pump inhibitor therapy. Recently, he has experienced episodes of sweating, tremulousness, and confusion that improve after eating.

Laboratory investigations reveal:

- Serum calcium: 11.8 mg/dL
- Serum PTH: elevated
- Fasting serum gastrin: markedly elevated
- Blood glucose during symptoms: 42 mg/dL
- Serum insulin: inappropriately elevated

The physician suspects Multiple Endocrine Neoplasia Type 1 (MEN 1).

Which of the following combinations of endocrine tumors is most characteristic of this syndrome?

- a) Medullary thyroid carcinoma, pheochromocytoma, parathyroid hyperplasia
- b) Pituitary adenoma, parathyroid adenoma, pancreatic endocrine tumor
- c) Pheochromocytoma, mucosal neuroma, medullary thyroid carcinoma
- d) Adrenal adenoma, thyroid follicular carcinoma, insulinoma

Q89. A 26-year-old woman with a 4-year history of Systemic Lupus Erythematosus (SLE) is planning her first pregnancy. She is currently in remission and is taking medications regularly. During a preconception counseling visit, the physician advises her to continue a particular drug throughout pregnancy because it reduces lupus flares and improves maternal and fetal outcomes.

Which of the following is the drug of choice that should be continued during pregnancy in this patient?

- a) Methotrexate
- b) Mycophenolate mofetil
- c) Hydroxychloroquine



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d) Cyclophosphamide

Q90. A 38-year-old right-handed school teacher complains that every morning she struggles to hold a toothbrush and button her shirt. By the time she reaches school, her symptoms have significantly improved. Over the last year, she has noticed progressive deformity of her fingers and increasing difficulty opening jars. Examination reveals symmetrical swelling of the MCP and PIP joints with ulnar deviation of the fingers. X-ray of the hands shows juxta-articular osteopenia and marginal erosions.

The underlying mechanism responsible for the irreversible joint destruction in this disease is the formation of which of the following structures?

- a) Rice bodies
- b) Tophi
- c) Pannus
- d) Heberden nodes

Q91. A 45-year-old woman presents with dry eyes, difficulty swallowing dry foods without water, and recurrent dental caries. On examination, there is reduced salivary pooling in the oral cavity.

Which of the following antibodies is most strongly associated with this condition?

- a) Anti-dsDNA antibody
- b) Anti-centromere antibody
- c) Anti-Ro (SS-A) antibody
- d) Anti-CCP antibody

Q92. A 72-year-old man presents with sudden onset pain, swelling, and redness of the right knee. Synovial fluid aspiration reveals calcium pyrophosphate crystal deposition disease.

Which of the following combinations of crystal morphology and birefringence is most characteristic of this condition?

- a) Needle-shaped crystals with negative birefringence
- b) Rhomboid-shaped crystals with positive birefringence
- c) Rhomboid-shaped crystals with negative birefringence
- d) Needle-shaped crystals with positive birefringence

Q93. A 20-year-old male medical student presents with recurrent episodes of mild jaundice, especially during examination periods and after prolonged fasting. He denies abdominal pain, pruritus, dark urine, weight loss, or alcohol use. Physical examination is unremarkable except for mild scleral icterus.



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Laboratory investigations reveal:

- Total bilirubin: 3.2 mg/dL
- Predominantly unconjugated hyperbilirubinemia
- Normal AST, ALT, and ALP
- Normal complete blood count and reticulocyte count
- No evidence of hemolysis

The physician reassures him that the condition is benign and requires no treatment.

Which of the following is the most likely underlying defect in this patient?

- a) Mutation in ABCC2 causing defective biliary excretion of conjugated bilirubin
- b) Complete absence of bilirubin UDP-glucuronyl transferase activity
- c) Reduced activity of bilirubin UDP-glucuronyl transferase (UGT1A1)
- d) Mutation in ATP7B causing impaired copper excretion

Q94. A 32-year-old woman presents with 3 months of fatigue, anorexia, and intermittent right upper quadrant discomfort. She reports joint pains and had an episode of jaundice last month. She has no history of alcohol use or medication toxicity. She has a past history of Hashimoto thyroiditis.

Physical exam shows mild hepatomegaly.

Labs:

AST: 620 U/L

ALT: 710 U/L

Total bilirubin: 3.0 mg/dL

IgG levels: markedly elevated

Viral hepatitis panel: negative

ANA: positive

Anti-smooth muscle antibody (SMA): strongly positive

Liver biopsy shows interface hepatitis with a plasma cell infiltrate.

What is the most likely diagnosis?

- a) Alcoholic hepatitis
- b) Autoimmune hepatitis
- c) Wilson disease
- d) Drug-induced liver injury



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Q95. A 32-year-old man with a long-standing history of ulcerative colitis presents with progressive fatigue, generalized pruritus, and intermittent jaundice. Laboratory investigations reveal a cholestatic pattern of liver injury with markedly elevated ALP and GGT. MRCP demonstrates multifocal strictures and dilatations of the intrahepatic and extrahepatic bile ducts, producing a characteristic "beaded" appearance.

Which of the following antibodies is most commonly associated with this patient's condition?

- a) Anti-mitochondrial antibody (AMA)
- b) Anti-smooth muscle antibody (ASMA)
- c) p-ANCA (perinuclear anti-neutrophil cytoplasmic antibody)
- d) Anti-liver kidney microsomal antibody (Anti-LKM-1)

Q96. A 52-year-old man presents with progressive fatigue, joint pain involving the second and third metacarpophalangeal joints, and decreased libido. On examination, his skin has a diffuse bronze pigmentation. Laboratory investigations reveal:

- Elevated serum ferritin
- Increased transferrin saturation (>45%)
- Mildly elevated liver enzymes
- Fasting blood glucose: 180 mg/dL

Ultrasound shows hepatomegaly, and liver biopsy demonstrates excessive iron deposition within hepatocytes. Mutation in which of the following genes is most commonly responsible for this condition?

- a) ATP7B
- b) HFE
- c) UGT1A1
- d) SERPINA1

Q97. A 65-year-old man is admitted to the intensive care unit with septic shock secondary to pneumonia. His baseline serum creatinine 1 month ago was 0.8 mg/dL. During hospitalization, his urine output progressively declines. On day 3, investigations reveal:

- Serum creatinine: 3.8 mg/dL
- Urine output: 0.2 mL/kg/hour for the past 24 hours

According to the KDIGO criteria for Acute Kidney Injury (AKI), what is the stage of AKI in this patient?

- a) Stage 1 AKI
- b) Stage 2 AKI
- c) Stage 3 AKI
- d) Does not meet criteria for AKI



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Q98. A 9-year-old boy is brought to the pediatric clinic with facial puffiness, cola-colored urine, and decreased urine output for the past 3 days. His mother reports that he had a sore throat about 2 weeks ago, which resolved without treatment.

On examination:

- Blood pressure: 140/90 mmHg
- Periorbital edema present

Laboratory investigations reveal:

- Urinalysis: Hematuria with red blood cell casts
- Mild proteinuria
- Serum creatinine: mildly elevated
- Serum C3 level: decreased
- Elevated anti-streptolysin O (ASO) titer

Which of the following is the most characteristic histopathological finding in this condition?

- a) Diffuse proliferation of glomerular cells with subepithelial “hump-shaped” immune deposits
- b) Podocyte foot process effacement on electron microscopy
- c) Thickened glomerular basement membrane with “spike and dome” appearance
- d) Nodular mesangial sclerosis (Kimmelstiel–Wilson nodules)

Q99. A 45-year-old man presents with enlargement of hands and feet, coarse facial features, increased spacing between teeth, and excessive sweating over the past 3 years. He also reports headaches and recent onset of visual field defects. Examination shows bitemporal hemianopia.

Laboratory investigations reveal:

- Elevated serum IGF-1 levels
- Failure of GH suppression after oral glucose tolerance test

MRI of the brain shows a pituitary macroadenoma. Which of the following is the drug of choice for medical management of this condition?

- a) Bromocriptine
- b) Octreotide
- c) Ketoconazole
- d) Mifepristone

Q100. A 32-year-old woman is brought to the emergency department with episodes of sweating, palpitations, tremors, irritability, and confusion, which occur mainly in the early morning and improve



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after eating food or receiving intravenous glucose. Her family reports that she has had similar intermittent episodes over the past few weeks.

During one such episode in the hospital, her capillary blood glucose is found to be 38 mg/dL, and her symptoms improve rapidly after dextrose infusion. Further evaluation is done to determine the cause of hypoglycemia.

Laboratory findings during the hypoglycemic episode show:

- Serum insulin: elevated
- Serum C-peptide: suppressed
- Sulfonylurea screen: negative

There is no history of diabetes mellitus, oral hypoglycemic drug use, or significant weight loss. Which of the following is the most likely cause of her hypoglycemia?

- a) Exogenous insulin administration
- b) Insulinoma
- c) Factitious hypoglycemia due to sulfonylurea intake
- d) Reactive (postprandial) hypoglycemia

Q101. A 23-year-old woman with known type 1 diabetes mellitus presents with vomiting, abdominal pain, and deep, rapid breathing. On examination, she appears dehydrated. Laboratory investigations reveal:

- Blood glucose: 580 mg/dL
- Arterial pH: 7.12
- Serum bicarbonate: 9 mEq/L
- Positive serum ketones

What is the most appropriate initial management step?

- a) Start subcutaneous long-acting insulin
- b) Begin IV fluids followed by IV insulin
- c) Give oral bicarbonate
- d) Give glucagon injection

Q102. A 23-year-old woman presents with facial puffiness, pedal edema, and decreased urine output for the past 2 weeks. She has a history of photosensitivity, recurrent oral ulcers, and joint pain involving both hands. Laboratory investigations reveal proteinuria (4.5 g/day), hematuria, elevated serum creatinine, positive ANA, and anti-dsDNA antibodies. A renal biopsy is performed. Light microscopy shows diffuse thickening of capillary walls with “wire-loop” lesions due to extensive subendothelial immune complex deposition.

Which class of lupus nephritis is most likely present in this patient?



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- a) Class II – Mesangial proliferative lupus nephritis
- b) Class III – Focal lupus nephritis
- c) Class IV – Diffuse proliferative lupus nephritis
- d) Class V – Membranous lupus nephritis

Q103. A 40-year-old woman presents with pain and swelling of the small joints of both hands for 6 months. She reports morning stiffness lasting more than an hour that improves with activity. On examination, there is symmetrical involvement of the MCP and PIP joints. The physician suspects Rheumatoid Arthritis (RA) and orders an antibody test as an initial screening investigation. Which of the following antibodies is most commonly used for screening Rheumatoid Arthritis?

- a) Anti-cyclic citrullinated peptide (Anti-CCP) antibody
- b) Rheumatoid factor (RF)
- c) Anti-double-stranded DNA (Anti-dsDNA) antibody
- d) Anti-Smith (Anti-Sm) antibody

Q104. A 32-year-old woman presents with Raynaud phenomenon, swollen fingers, pain in multiple small joints of the hands, and progressive difficulty swallowing. Examination reveals puffy hands and mild skin tightening over the fingers. Laboratory investigations show positive ANA. The clinician suspects Mixed Connective Tissue Disease (MCTD) due to overlapping features of systemic sclerosis, SLE, and polymyositis. Which of the following antibodies is most characteristic of this condition?

- a) Anti-centromere antibody
- b) Anti-dsDNA antibody
- c) Anti-U1 ribonucleoprotein (U1-RNP) antibody
- d) Anti-Scl-70 antibody

Q105. 58-year-old man presents with recurrent episodes of severe pain and swelling in the first metatarsophalangeal joint for the past 10 years. He now has multiple firm nodules over the helix of the ear and the extensor surface of the elbow. Fine-needle aspiration from one of the nodules reveals needle-shaped, negatively birefringent monosodium urate crystals. His serum uric acid level is 9.8 mg/dL, and renal function is normal. The physician decides to start a drug that lowers serum uric acid by increasing its renal excretion.

Which of the following drugs acts by this mechanism?

- a) Allopurinol
- b) Febuxostat
- c) Probenecid
- d) Colchicine



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- Q106. A 33-week preterm baby has feeble cry, sluggish movements and irregular breathing immediately after birth. Positive pressure ventilation is being given and chest compression is planned. What heart-rate cut-off indicates chest compression?
- HR < 160/minute
 - HR < 120/minute
 - HR < 100/minute
 - HR < 60/minute
- Q107. A 1.5-year-old child has distended abdomen, flaky-paint skin, patches of hair loss, irritability and refusal of food. Weight-for-height is below 2 SD. What is the most likely diagnosis?
- Marasmus
 - Kwashiorkor
 - Marasmic-kwashiorkor
 - Chronic malnutrition
- Q108. Which is the best marker for establishing the diagnosis of stunting in children?
- Weight for height
 - Mid-arm circumference
 - Height for age
 - Weight for age
- Q109. In severe acute malnutrition, iron supplementation is avoided in the initial week of management because:
- Iron supplementation increases risk of constipation
 - Malnourished children are allergic to iron initially
 - Iron should not be administered with IV fluids
 - It can promote bacterial growth and increase infections
- Q110. Which among these milestones is the last to develop in a child?
- Stranger anxiety
 - Babbling
 - Speaking jargons
 - Recognizing the mother
- Q111. An infant is evaluated for development. Which statement about the postural reflex shown in the image is incorrect?



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- a) Appears after birth
- b) Never disappears
- c) Protective reflex
- d) Commonly seen in infants between 3-6 months of age

Q112. A 32-week baby has poor breathing efforts, grunting and nasal flaring soon after birth. X-ray shows low lung volume, ground-glass opacity and air bronchograms; ABG shows severe acidosis and respiratory failure. What is the initial management?

- a) Start 100% oxygen by mask
- b) Start IV ceftriaxone, IV 10% dextrose and observe
- c) Start CPAP
- d) Surfactant through ET tube, start mechanical ventilation

Q113. A newborn has increasing tachypnea, cyanosis and a loud single S2. Chest X-ray is shown. What is the likely diagnosis?



- a) TOF
- b) Coarctation of aorta
- c) TGA
- d) TAPVR

Q114. Which finding is unlikely in examination of a preterm newborn?

- a) Red flushed skin
- b) Plenty of lanugo
- c) Deep sole creases of foot
- d) Slow elastic recoil of ears



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- Q115. A 3-day-old newborn is delivered to a mother with poorly controlled type 2 diabetes. Which congenital heart disease is most specific to maternal diabetes?
- TGA
 - VSD
 - ASD
 - Hypoplastic left heart syndrome
- Q116. A term neonate has a large fleshy sac from the lower spine containing meninges and spinal cord. In 2-3 days, progressive head enlargement and bulging fontanelles occur. In which malformation can both findings be seen?
- Dandy Walker syndrome
 - Arnold Chiari malformation
 - Neural tube defect
 - Dolicocephaly
- Q117. Which drug is the most appropriate front-line medication for long-term symptom control/maintenance therapy in childhood asthma?
- Inhaled corticosteroids
 - Systemic corticosteroids
 - SABA
 - LABA
- Q118. A 2-year-old child is evaluated for growth. Most 2-year-old children achieve a length/height of about:
- 50 cm
 - 85 cm
 - 100 cm
 - 120 cm
- Q119. A 29-week preterm baby had severe birth asphyxia, respiratory difficulty and sepsis. Imaging shows bilateral multiple cystic lesions in periventricular white matter, diagnosed as PVL. Which cerebral palsy is most likely?
- Spastic diplegia
 - Spastic hemiplegia
 - Spastic quadriplegia
 - Dyskinetic cerebral palsy



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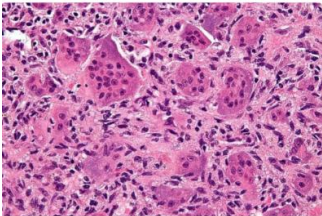
Q120. Most common site of intracranial bleeding in a preterm baby is:

- a) Intraventricular
- b) Basal ganglia
- c) Brainstem
- d) Germinal matrix

Q121. A 20-year-old man presents with chronic back pain. The pain is worse in the morning and reduces with physical activity within 1.5 hours. He has a history of anterior uveitis. X-ray of the sacroiliac joint is normal. On examination, lumbar lordosis is lost, and spinal flexion is reduced. What is the diagnostic modality for early diagnosis of this condition?

- a) X-ray of dorsolumbar spine
- b) Anti-CCP antibody
- c) CT scan of sacroiliac joint
- d) MRI of sacroiliac joint

Q122. A 30-year-old woman develops a tumor around the knee joint. Biopsy shows numerous multinucleated osteoclast-like giant cells scattered among mononuclear stromal cells. What is the most likely diagnosis?



- a) Osteosarcoma
- b) Ewing sarcoma
- c) Giant cell tumor
- d) Osteoblastoma

Q123. Identify the prosthesis shown in the image.



- a) Austin-Moore prosthesis
- b) Thompson prosthesis



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- c) Jepson hip prosthesis
- d) Bohlmann prosthesis

Q124. An elderly woman presents with backache, fatigue, low mood, hypercalcemia, and renal stones. X-ray of the hand shows subperiosteal bone resorption along the radial aspect of the middle phalanges. What is the most probable diagnosis?



- a) Multiple myeloma
- b) Metastatic disease
- c) Hyperparathyroidism
- d) Osteomalacia

Q125. A 16-year-old boy presents with pain and swelling around the lower end of the femur. X-ray shows a mixed lytic-sclerotic lesion in the metaphysis with a sunburst appearance and Codman's triangle. What is the most likely diagnosis?

- a) Ewing sarcoma
- b) Osteoclastoma
- c) Osteosarcoma
- d) Chondrosarcoma

Q126. Apocrine glands are found in which of the following areas?

- a) Scalp
- b) Axilla
- c) Palms and soles
- d) Face

Q127. An HIV patient presents with a white corrugated plaque on the tongue that cannot be rubbed off. What is the most likely diagnosis?

- a) Candida albicans
- b) HSV
- c) HHV-8

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d) EBV

Q128. A young male complains of an itchy rash over his back as shown. What is the most likely skin lesion and diagnosis?



- a) Collarette scale - Pityriasis rosea
- b) Wickham striae - Lichen planus
- c) Christmas tree pattern - Psoriasis
- d) Woronoff ring - Psoriasis

Q129. An overweight 12-year-old girl has blackish discoloration on her neck as shown. What is the condition and its association?



- a) Acanthosis nigricans - Insulin resistance
- b) Stria albicans - Cushing syndrome
- c) Xeroderma pigmentosum - Malignancy
- d) Cafe-au-lait spots - McCune-Albright syndrome

Q130. A middle-aged Indian man comes with the facial lesion shown. What is the most likely diagnosis?



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- a) Basal cell carcinoma
- b) Squamous cell carcinoma
- c) Lupus vulgaris
- d) Nevus

Q131. A patient with schizophrenia first heard the voice of God as her clock ticked; later voices came from running tap water and chirping birds. This is an example of:

- a) Illusion
- b) Extracampine hallucination
- c) Reflex hallucination
- d) Functional hallucination

Q132. A chronic alcoholic blames his family environment as the cause of his alcoholism. This is:

- a) Projection
- b) Denial
- c) Rationalization
- d) Sublimation

Q133. A 35-year-old woman hears voices for 7 months. When asked the proverb "money does not grow on trees", she replies "money grows in Reserve Bank". What mental-status abnormality is present?

- a) Abstract thinking
- b) Flight of ideas
- c) Delusion
- d) Illusion

Q134. A 20-year-old girl has unstable relationships, multiple breakups, and cuts her wrist after each breakup. What kind of personality pattern is suggested?

- a) Dependent
- b) Borderline pattern

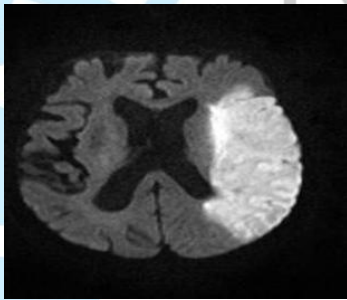
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- c) Histrionic
- d) Anancastic

Q135. A patient with bipolar disorder has episodes of mania and depression but poor compliance with drugs. Which psychotherapy is preferred?

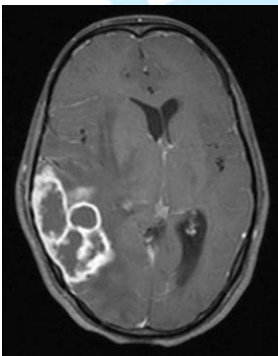
- a) CBT
- b) DBT
- c) Psychoeducation
- d) Behaviour therapy

Q136. A 65-year-old man presents with right-sided hemiparesis. Imaging is shown. All of the following statements are true except:



- a) CT done in the first few hours may be normal
- b) The image is MRI showing acute infarct
- c) Thrombolysis is the treatment of choice in the early window
- d) Large vessel occlusion is determined on MRI brain

Q137. A child has fever, altered sensorium and seizure. MRI is shown. What is the diagnosis?



- a) Tuberculoma
- b) Neurocysticercosis



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- c) Abscess
- d) Neuroblastoma metastasis

Q138. Sestamibi scan is used for localization of which tumour?

- a) Carcinoid tumour
- b) Parathyroid adenoma
- c) Pheochromocytoma
- d) Anaplastic thyroid cancer

Q139. Lutetium-177 PSMA is used in treatment of which condition?

- a) Metastatic pancreatic cancer
- b) Metastatic breast cancer
- c) Metastatic prostate cancer
- d) Metastatic colon cancer

Q140. All are true about ultrasonography except:

- a) Economical and widely available
- b) Non-subjective
- c) Can be used in diagnosis of pneumothorax
- d) POCUS stands for Point-of-Care USG

Q141. Investigation of choice for diagnosis of osteoid osteoma of femur is:

- a) Radiography
- b) CE-MRI
- c) NCCT
- d) PET scan

Q142. All are true about X-ray production except:

- a) Cathode and anode are made of tungsten
- b) LINAC produces only photon
- c) Compton effect results in scatter radiation
- d) Thermionic emission produces electrons at cathode

Q143. All are true about chest X-ray except:

- a) Right heart border is formed by SVC, right atrium and right ventricle
- b) Left atrium forms the base of heart



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- c) Right ventricle forming the apex is always abnormal
- d) Most dependent pleural recess in erect position is seen on lateral CXR

Q144. The abnormality shown in the IVP image is seen in which condition?



- a) Polycystic kidney disease
- b) Horseshoe kidney
- c) Duplex renal system
- d) Ureterocele

Q145. A known case of diverticulosis presents with left iliac fossa pain, fever and chills. Which investigation is contraindicated?

- a) Barium meal follow through
- b) Colonoscopy
- c) USG
- d) CECT abdomen

Q146. During resuscitation, a patient received 20% intralipid injection. What was the most probable diagnosis?

- a) Local anaesthesia toxicity
- b) Asphyxia due to drowning
- c) Electrocution
- d) Pulseless electrical activity

Q147. During laparoscopic surgery, EtCO₂ suddenly falls from 36 to 22 mmHg while PaCO₂ is 48 mmHg. BP drops and heart rate rises. What best explains this?

- a) Increased physiological dead space due to pulmonary embolism
- b) Decreased CO₂ production due to hypothermia
- c) Increased alveolar ventilation due to hyperventilation
- d) Rebreathing of CO₂ due to faulty circuit



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- Q148. A 68-year-old diabetic man with stage 4 CKD and deranged LFTs needs a nondepolarizing neuromuscular blocker that degrades spontaneously in plasma and does not rely on liver or kidney metabolism. Which agent is selected?
- a) Vecuronium
 - b) Cisatracurium
 - c) Rocuronium
 - d) Pancuronium
- Q149. A 64-year-old diabetic hypertensive man scheduled for total hip arthroplasty had MI one year ago and now has chest pain on climbing three flights of stairs but is comfortable at rest. What ASA class is he?
- a) ASA III
 - b) ASA IV
 - c) ASA V
 - d) ASA VI
- Q150. The image shows a portable inhalational analgesia cylinder used in labour with a demand valve. What is the gas mixture?



- a) 70% N₂O + 30% O₂
- b) 60% N₂O + 40% O₂
- c) 50% N₂O + 50% O₂
- d) 100% N₂O

ANSWERS

1	b
2	a
3	c

4	b
5	c
6	c

7	a
8	a
9	c

10	a
11	c
12	a

13	c
14	c
15	a



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16	b
17	b
18	b
19	b
20	b
21	b
22	c
23	b
24	c
25	b
26	b
27	b
28	b
29	c
30	b
31	b
32	b
33	b
34	b
35	b
36	b
37	d
38	c
39	b
40	c
41	c
42	c

43	b
44	c
45	b
46	b
47	b
48	b
49	c
50	c
51	c
52	c
53	c
54	c
55	c
56	a
57	d
58	b
59	b
60	c
61	a
62	a
63	b
64	c
65	c
66	a
67	a
68	b
69	a

70	c
71	c
72	b
73	c
74	c
75	d
76	c
77	c
78	b
79	c
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134	b
135	c
136	d
137	a
138	b
139	c
140	b
141	c
142	b
143	a
144	c
145	b
146	a
147	a
148	b
149	a
150	c

NEXT

